

Colon & Colorectal Cancer — Clinical Trial & Treatment Timeline

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Clinical Summary

Recent developments in colorectal cancer research highlight several promising clinical trials targeting advanced stages of the disease. Notably, trials are underway for patients with Stage IV colorectal cancer, including studies on combination therapies such as Tegavivint, dual-target CAR-NK cells, and various immunotherapy regimens. These advancements may provide new treatment options for patients with specific biomarkers or unresectable disease, potentially influencing treatment decisions. Moving forward, it will be important to monitor the outcomes of these trials, particularly those involving novel combinations and targeted therapies.

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Situation Summary

Events Detected: 10
High Significance: 0
Top Category: New Trial
Primary Stage: Stage IV
Primary Target: General
Research Activity: Active Monitoring

Event Timeline

EVENT CLUSTER — Combination of High and Low-Dose Radiotherapy With Immune Therapy and TKI in Advanced Colorectal Cancer: A Phase II Study

MEDIUM **STAGE IV** **GENERAL** **NEW TRIAL** **VERIFIED** **PUBLISHED MAR 12**

Mar 12 12:11 · immediate · [clinicaltrials.gov](#)

A new Phase II study is exploring the effectiveness of combining high and low-dose radiotherapy with immune therapy and targeted therapy in advanced colorectal cancer, potentially offering patients new treatment options to enhance their outcomes.

1 source(s) reporting · cluster 1 of 10

EVENT CLUSTER — Safety and Efficacy of Tegavivint in Patients With Metastatic Colorectal Carcinoma

MEDIUM

STAGE IV

GENERAL

NEW TRIAL

VERIFIED

PUBLISHED MAR 11

Mar 12 12:11 · immediate · clinicaltrials.gov

A new clinical trial is evaluating the safety and effectiveness of Tegavivint for patients with advanced metastatic colorectal cancer, potentially offering a new treatment option for this challenging stage of the disease.

1 source(s) reporting · cluster 2 of 10

EVENT CLUSTER — Dual-Target CAR-NK Cells for Biomarker-Selected Advanced Colorectal Cancer

MEDIUM

STAGE IV

PD-1/PD-L1

NEW TRIAL

VERIFIED

PUBLISHED MAR 10

Mar 12 12:11 · immediate · clinicaltrials.gov

A new clinical trial is exploring the use of dual-target CAR-NK cells to treat advanced colorectal cancer, potentially offering a promising treatment option for patients whose tumors express specific biomarkers.

1 source(s) reporting · cluster 3 of 10

EVENT CLUSTER — A Phase II/III Clinical Trial of RC148 Plus Chemotherapy as 1L Therapy for Unresectable or Metastatic Colorectal Cancer

MEDIUM

STAGE IV

GENERAL

NEW TRIAL

VERIFIED

PUBLISHED MAR 10

Mar 12 12:11 · immediate · clinicaltrials.gov

A new clinical trial is investigating the combination of RC148 and chemotherapy as a first-line treatment for patients with unresectable or metastatic colorectal cancer, potentially offering a new option for improving outcomes in this advanced stage of the disease.

1 source(s) reporting · cluster 4 of 10

EVENT CLUSTER — Low-Dose Radiotherapy to Sensitize Pucotenlimab Plus CAPEOX for pMMR Locally Advanced Rectal Cancer

MEDIUM

STAGE III

GENERAL

NEW TRIAL

VERIFIED

PUBLISHED MAR 4

Mar 12 12:11 · immediate · clinicaltrials.gov

A new clinical trial is exploring the use of low-dose radiotherapy to enhance the effectiveness of the immunotherapy drug pucotenlimab combined with CAPEOX chemotherapy for patients with locally advanced rectal cancer, potentially offering a more effective treatment option.

1 source(s) reporting · cluster 5 of 10

EVENT CLUSTER — A Clinical Study of Iparomlimab and Tuvonralimab Combined With Bevacizumab and Alternating Triweekly CAPOX/mCAPIRI Regimen as First-line Treatment for Unresectable Advanced Colorectal Cancer

MEDIUM

STAGE IV

VEGF

NEW TRIAL

VERIFIED

PUBLISHED MAR 3

Mar 12 12:11 · immediate · clinicaltrials.gov

A new clinical trial is exploring a combination of immunotherapy and chemotherapy for patients with advanced colorectal cancer, potentially offering a promising first-line treatment option for those with unresectable disease.

1 source(s) reporting · cluster 6 of 10

EVENT CLUSTER — Everolimus in CDK12-Deficient Metastatic Colorectal Cancer (EVER-RECODE)

MEDIUM

STAGE IV

GENERAL

NEW TRIAL

VERIFIED

PUBLISHED FEB 27

Mar 12 12:11 · immediate · clinicaltrials.gov

The EVER-RECODE trial is exploring the potential of everolimus to improve treatment outcomes for patients with Stage IV colorectal cancer that has CDK12 deficiencies, offering a new avenue for therapy in this challenging disease.

1 source(s) reporting · cluster 7 of 10

EVENT CLUSTER — FOLFOX Chemotherapy Combined With Fruquintinib and Serplulimab as First-Line Conversion Therapy for Initially Unresectable pMMR/MSS Colorectal Cancer

MEDIUM

STAGE IV

GENERAL

NEW TRIAL

VERIFIED

PUBLISHED MAR 3

Mar 12 12:11 · immediate · clinicaltrials.gov

A new clinical trial is exploring the combination of FOLFOX chemotherapy with Fruquintinib and Serplulimab as a promising first-line treatment for patients with initially unresectable colorectal cancer, potentially improving options for those with this challenging diagnosis.

1 source(s) reporting · cluster 8 of 10

EVENT CLUSTER — Neoadjuvant Chemoradiotherapy Plus Tislelizumab With or Without Probio-M9 in pMMR/MSS Locally Advanced Rectal Cancer

MEDIUM

STAGE III

GENERAL

NEW TRIAL

VERIFIED

PUBLISHED MAR 10

Mar 12 12:11 · immediate · clinicaltrials.gov

A new clinical trial is exploring the combination of neoadjuvant chemoradiotherapy with the immunotherapy drug Tislelizumab, potentially offering improved treatment options for patients with locally advanced rectal cancer.

1 source(s) reporting · cluster 9 of 10

EVENT CLUSTER — FOLFIRI and Bevacizumab With or Without Pelareorep for Second-Line Treatment of Metastatic RAS-Mutated, Microsatellite-Stable Colorectal Cancer

MEDIUM

STAGE IV

VEGF

NEW TRIAL

VERIFIED

PUBLISHED MAR 4

Mar 12 12:11 · immediate · clinicaltrials.gov

A new clinical trial is exploring whether adding Pelareorep to the standard treatment of FOLFIRI and Bevacizumab can improve outcomes for patients with advanced RAS-mutated, microsatellite-stable colorectal cancer.

1 source(s) reporting · cluster 10 of 10

Appendix A — Patient-Friendly Summary

Plain-language overview for patients, caregivers, and family members

What's happening

Recently, there have been several exciting developments in the treatment of advanced colorectal cancer. A new study is looking at combining high and low-dose radiotherapy with immune therapy and targeted therapy, which could provide new options for patients. Other trials are testing new drugs like Tegavivint and exploring the use of CAR-NK cells, which are designed to target specific features of tumors. There are also trials investigating combinations of chemotherapy with immunotherapy, which may help improve outcomes for those with advanced disease. These studies are important as they could lead to new treatments that work better for patients.

What this means for you

For patients, these developments mean there may be new options on the horizon. If you or a loved one is dealing with advanced colorectal cancer, it's a good idea to talk to your oncologist about these new trials. Ask questions about whether you might be eligible and what the potential benefits could be. Stay informed about the latest research, as it could open doors to treatments that are more effective. Remember, you are not alone in this journey, and there are many people working hard to find better ways to help you.

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This is not medical advice. Always speak with your oncologist before making any treatment decisions.

Appendix B — Colon & Colorectal Cancer Treatment Reference

Incidence & Trends

~150,000 new US cases/year · ~52,000 deaths/year · 4th most common US cancer

▲ Early-Onset Alert: CRC rates in adults under 50 have risen ~2% per year since the 1990s · Adults under 55 now account for ~20% of all new CRC diagnoses · Cause under active investigation (diet, microbiome, obesity, sedentary lifestyle)

Current Screening Guidelines (2025)

USPSTF / ACS / ACG — Average Risk: Begin screening at age 45 (lowered from 50 in 2021)

Colonoscopy every 10 years · FIT (fecal immunochemical test) annually · Cologuard every 1-3 years · CT colonography every 5 years

High Risk (family history / Lynch syndrome / IBD): Begin at age 40 or 10 years before youngest affected relative — whichever comes first · Colonoscopy every 1-5 years depending on risk

Symptoms at any age: Rectal bleeding, change in bowel habits >4 weeks, unexplained weight loss, iron deficiency anemia → immediate evaluation regardless of age

Key Biomarkers — Test All mCRC at Diagnosis

RAS (KRAS/NRAS) · BRAF V600E · MSI/MMR status · HER2 · NTRK fusion · TMB

Stage IV Targeted Options

KRAS G12C (~3-4%) → sotorasib + cetuximab · adagrasib + cetuximab

BRAF V600E (~8-10%) → encorafenib + cetuximab ± binimetinib

MSI-H/dMMR (~5%) → pembrolizumab (1st line preferred) · nivolumab ± ipilimumab

HER2 amplified (~2-3%) → trastuzumab + pertuzumab · trastuzumab deruxtecan

RAS/BRAF WT → FOLFOX or FOLFIRI + cetuximab or panitumumab (left-sided preferred)

1st Line mCRC Standard of Care

FOLFOX or FOLFIRI ± bevacizumab (RAS mutant / right-sided)

FOLFOX or FOLFIRI + cetuximab or panitumumab (RAS/BRAF WT, left-sided)

Pembrolizumab monotherapy (MSI-H/dMMR)

Key Resources

ClinicalTrials.gov · NCCN Guidelines (nccn.org) · Colorectal Cancer Alliance (ccalliance.org) · Fight Colorectal Cancer (fightcrc.org) · ACS (cancer.org)

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